

Pediatric P-05 Pediatric Tachycardia

Stabilize airway using BLS adjuncts if necessary
Oxygen – High Flow via NRB
 Ventilate using BVM if necessary
 Vital signs
 Pulse Oximetry
 Cardiac Monitor
 IV/IO Access
 Consider **NS** 20mL/kg bolus for hypotension

Narrow Complex
Tachycardia?

Probable Sinus Tachycardia

- P waves present and normal
- Variable R-R and constant P-R
- Infant HR usually <220
- Child HR <180

Treat underlying cause

Monitor and reassess

Probable SVT

- P waves absent or abnormal
- HR not variable
- Infant HR ≥220
- Child HR ≥180

Vagal Maneuver
If no cardiopulmonary compromise

Vagal Maneuver
Successful?

Yes

No

Hypotension and
shock?

No

Yes

Adenosine 0.1mg/kg rapid IV/IO
Max single dose 6mg

If no response to first Adenosine dose

Adenosine 0.2mg/kg rapid IV
Max single dose 12mg

Synchronized Cardioversion
0.5 – 1J/kg
If not effective, increase to 2J/kg

Wide Complex
Tachycardia?

Probable VT

Hypotension and
shock?

No

Yes

Synchronized Cardioversion
0.5 – 1J/kg
If not effective, increase to 2J/kg

Contact base hospital for
further treatment consultation

Signs and symptoms of shock
 ALOC, hypotension (age appropriate),
 capillary refill >2 seconds, diaphoresis, SOB

Consider prior to cardioversion

Refer to P-14 Pediatric Pain
Management for Fentanyl dose

For sedation:

Midazolam 0.1mg/kg IV/IO
may repeat in 2 minutes
OR

Midazolam 0.2mg/kg IM/IN
may repeat in 2 minutes

DISRUPTED COMMUNICATIONS

In the event of a "disrupted communications" situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.